Better Assessment Better Care

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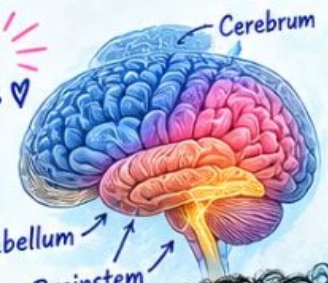
LEARN PRACTICE EXCEL

PATIENT ASSESSMENT IN NEUROLOGICAL CONDITIONS

THE COMPLETE STUDY GUIDE

FOR MBBS | BHMS | B.Sc. NURSING | BDS | BPT

Systematic Approach Better Diagnosis



- History Taking
- Mental Status
- Cranial Nerves
- Motor & Sensory
- Coordination
- Reflexes
- Gait & Balance









Small Steps Big Impact



Neurology Made Simple

KEY AREAS

- History Taking
- General Examination
- Mental Status Examination
- Cranial Nerve Examination
- Motor System
- Sensory System
- Cerebellar Function
- Reflexes
- Gait & Balance
- Special Tests
- Functional Assessment

WHY THIS BOOK?

- High-Yield Points
- Concept Clarity
- Diagrams & Flowcharts
- Previous Year Exam Focus
- Easy to Revise

MENTAL STATUS EXAMINATION | CRANIAL NERVES | MOTOR & SENSORY SYSTEM | COORDINATION & BALANCE | REFLEXES | GAIT ANALYSIS

For Medical Students & Future Healthcare Professionals

@restore_healthphysio

Better Assessment Better Care

PATIENT ASSESSMENT IN NEUROLOGICAL CONDITIONS
MBBS
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Patient Assessment in neurological condition

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OFFICIAL DIGITAL STUDY GUIDE VERIFICATION

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STUDY GUIDE DETAILS & CONTENTS:

Comprehensive high-yield notes designed for Bachelor of Physiotherapy (BPT), MBBS, BDS, and allied healthcare students. Covers fundamental anatomy, clinical assessments, pathological conditions, evidence-based physical therapy rehabilitation protocols, and exam-oriented high-yield review.

KEY HIGHLIGHTS:

- Exam-Focused: Concise points curated from leading medical textbooks and clinical references.
- Visual Diagrams: High-yield diagrams and tables for quick revision.
- Mobile & Tablet Optimized: Readable on phones, tablets, iPads, and laptops.
- Official Medicos Minds Release: Verified by Dr. MD Kamran Alam.

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1. Introduction :-

- Axillary nerve is a branch of posterior cord of brachial plexus (C5-C6).
- It supplies deltoid and teres minor muscles and provides sensory supply to skin over the lateral aspect of shoulder (regimental badge area).
- Injury to axillary nerve leads to weakness of shoulder abduction and loss of sensation over lateral shoulder.

2. Anatomy :-

a) Origin :-

- Formed by fibers from posterior cord of brachial plexus (C5-C6).
- Lies posterior to subscapular nerve.

b) Course :-

- Passes through quadrangular space (with posterior circumflex humeral artery).
- Winds around surgical neck of humerus.
- Runs with posterior circumflex humeral vessels.
- Divides into anterior and posterior branches.

c) Supply :-

- Motor - Deltoid and teres minor muscles.
- Sensory - Skin over lateral aspect of shoulder (regimental badge area).

3. Causes :-

- Fracture of surgical neck of humerus.
- Dislocation of shoulder (anterior / posterior).
- Compression during operation (e.g. shoulder surgery, lithotomy position).
- Penetrating injury.
- Excessive traction on arm.
- Neuralgic amyotrophy (Parsonage-Turner syndrome).

4. Clinical Features :-

a) Motor signs :-

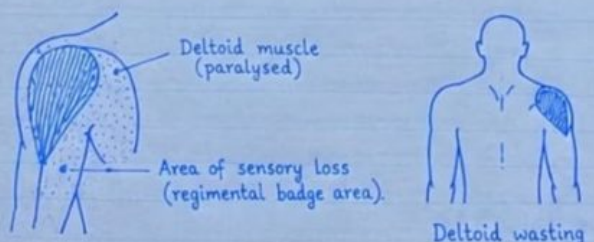
- Weakness / paralysis of deltoid muscle.
- Loss of power of shoulder abduction (0-15°).
- Difficulty in lifting arm away from body.
- Winging of scapula (due to deltoid weakness).

b) Sensory signs :-

- Loss of sensation over lateral aspect of shoulder (regimental badge area).

c) Physical examination :-

- Observe contour of shoulder (flattening of deltoid).
- Test active and passive abduction.
- Examine sensory area (regimental badge area).



5. Differential Diagnosis :-

- Suprascapular nerve palsy (affects supraspinatus and infraspinatus).
- Musculocutaneous nerve palsy (affects biceps).
- C5-C6 radiculopathy.
- Rotator cuff tear (pain and limited movement).
- Axillary artery injury (in addition to nerve palsy).

6. Management :-

a) Conservative :-

- Immobilization of shoulder in adduction and internal rotation (sling).
- Analgesics and anti-inflammatory drugs.
- Physiotherapy -
  - Passive and active assisted ROM exercises.
  - Isometric exercises for deltoid and rotator cuff.
  - Gradual strengthening (when recovery starts).
  - Prevention of shoulder stiffness and contractures.

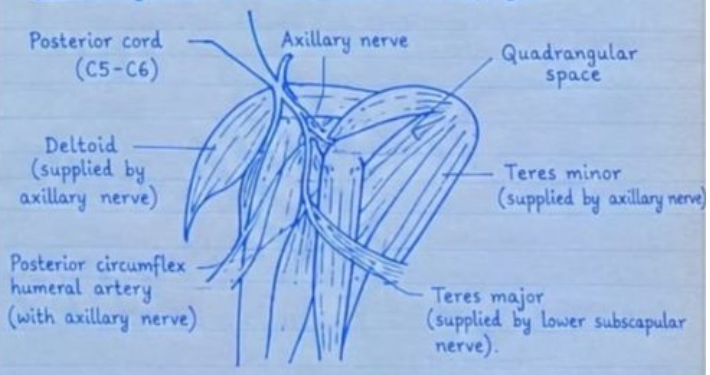
b) Surgical :-

- Indicated if no recovery in 3-6 months (depending on cause and severity).
- Nerve exploration and repair / grafting.
- Tendon transfer (e.g. latissimus dorsi or teres major to deltoid) in chronic cases.

7. Prognosis :-

- Good in neurapraxia and mild axonotmesis.
- Poor in severe injury or if associated with other nerve damage.
- Early physiotherapy improves functional outcome.

Axillary Nerve - Relation & Supply :-



8. Key Points :-

- Axillary nerve = C5-C6 (posterior cord).
- Supplies deltoid + teres minor.
- Motor loss → shoulder abduction weakness.
- Sensory loss → regimental badge area.
- Commonly injured in fracture / dislocation of humerus.
- Management → immobilization + physiotherapy, surgery if no recovery.
- Prognosis depends on severity and cause.

THANK YOU FOR YOUR PURCHASE!

We wish you the very best in your medical and physiotherapy examinations.

For any questions, supplementary questions, or study support, reach out to us:

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